

Arkansas Heart Hospital
1701 South Shackleford Road
Little Rock, AR 72211-4335

Patient: **MONSON, BERDIA T**
MRN: 00213113 Date of Service: 5/31/2026
FIN: 3650554 Provider: Hinson MD,Andrew
DOB/Age/Sex: 5/1/1937 89 years Female

Documentation

Note Type: History and Physical
Title: Admission H & P
Service Date/Time: 6/1/2026 11:17 CDT
Status: Auth (Verified)
Sign Information: Hinson MD,Andrew (6/1/2026 14:10 CDT); Meziere,APRN,
Crystal (6/1/2026 11:33 CDT)

Internal Provenance Source: Arkansas Heart Hospital; Hinson MD, Andrew

Chief Complaint

Weakness

History of Present Illness

89y/o with CABG, HTN, CHF, uterine cancer (completed radiation 1-2 weeks ago. now on immunotherapy). Presented to ED with c/o generalized weakness. progressively worsened over the past 3 days. Associated s/sx: lowBP, BLE edema, DOE, dry cough, wheezing. Recently held BP meds by recommendation of her cardiologist Dr. Lee Davis due to hypotension. Patient is on 2 L nasal cannula O2 at night (prescribed by her cardiologist). She denies h/o lung disease. Denies fever, chills, syncope, palliations, ABD pain, chest pain, melena, hematemesis.

In ER, noted to be hypoxic on room air. Lab work illustrates anemia 6.9/21.6. proBNP elevate at 4421. Creatinine elevated 2.06. CXR noted stable cardiomegaly and mild pulmonary vascular congestion. CT ABD/pelvis noted Third spacing due to fluid overload. This includes small bilateral pleural effusions, mild ascites, and subcutaneous anasarca and chronic findings (see diagnostic imaging below). 2 units PRBC ordered. Admitted to internal medicine for further treatment and evaluation.

Review of Systems

Review of Systems

Constitutional: no fever, no chills, no sweats, severe weakness

Skin: no Jaundice, no rash, no lesions, no petechiae

ENMT: no ear pain, no sore throat, no congestion, no hoarseness

Respiratory: moderate shortness of breath, moderate cough, moderate orthopnea, moderate wheezing

Cardiovascular: no chest pain, no palpitations, moderate edema

Gastrointestinal: no nausea, no vomiting, no diarrhea, no GI bleeding, no abdominal pain

Genitourinary: no dysuria, no hematuria, no discharge, no pain

Musculoskeletal: no back pain, no trauma

Neurologic: no headache, no dizziness, no numbness, no muscle weakness, no recent vision changes

Additional ROS info: Except as noted in the above Review of Systems and in the History of Present Illness all other systems have been reviewed and are negative or noncontributory.

Problem List/Past Medical History

Ongoing

CAD - Coronary artery disease

CHF - Congestive heart failure

History of cancer

HTN - Hypertension

Procedure/Surgical History

- CABG - Coronary artery bypass graft

infusaport placement

Medications

Inpatient

acetaminophen, 650 mg= 2 tab, Oral, every 4 hr, PRN

aspirin, 81 mg= 1 tab, Oral, Daily

docusate, 100 mg= 1 cap, Oral, Daily

furosemide(Lasix), 40 mg= 4 mL, IV Push, One Time

ipratropium-albuterol(DuoNeb 0.5 mg-2.5 mg/3 mL inhalation solution), 3 mL, NEB, QID, PRN

magnesium hydroxide(Milk of Magnesia 8% oral suspension), 30 mL, Oral, every 6 hr, PRN

melatonin, 3 mg= 1 tab, Oral, Daily at bedtime, PRN

nitroglycerin, 0.4 mg= 1 tab, Sublingual, every 5 min INT, PRN

ondansetron, 4 mg= 1 tab, Oral, every 4 hr, PRN

ondansetron, 4 mg= 2 mL, IV Push, every 4 hr, PRN

promethazine, 6.25 mg= 0.5 tab, Oral, every 6 hr, PRN

promethazine, 6.25 mg= 0.5 supp, Rectal, every 6 hr, PRN

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Physical Exam

Vitals & Measurements

T: 37.0 °C (Skin) TMIN: 36.0 °C (Axillary) TMAX: 37.2 °C (Skin) HR: 64 (Peripheral)

RR: 12 BP: 131/62 SpO2: 94% WT: 89.3 kg

General: Well developed, well nourished, in no acute distress

Head: Normocephalic, atraumatic

Lungs: on 4L nasal cannula. bilateral expiratory wheezes. BLL diminished with fine crackles

Heart: Regular rate, regular rhythm

Abdomen: Soft, non-tender, non-distended

Skin: Skin is pale, warm and dry

Psychiatric: Mood and affect normal and appropriate

Neurologic: Awake, alert, and oriented X3

extremities: BLE edema

Assessment/Plan

1. Weakness R53.1

secondary to anemia

2. Anemia D64.9

s/p 2units PRBC. 40mg lasix after each unit
recheck CBC in am

3. Renal insufficiency N28.9

could be secondary to fluid overload or recent hypotension
recheck labs in am
Hold ARB/ACEi

4. History of cancer Z85.9

uterine s/p radiation 1-2weeks ago
now on immunotherapy

5. CHF - Congestive heart failure I50.9

followed by Dr. lee davis
IV diuretics

6. CAD - Coronary artery disease I25.10

7. HTN - Hypertension I10

resume home meds as tolerated

Orders:

furosemide(Lasix), 40 mg= 4 mL, IV Push, One Time

ipratropium-albuterol(DuoNeb 0.5 mg-2.5 mg/3 mL inhalation solution), 3 mL, NEB, QID,
PRN

sodium biphosphate-sodium

phosphate(Fleet Enema 19 g-7 g/118 mL
rectal enema), 133 mL, Rectal, every 6
hr, PRN

sodium chloride(sodium chloride 0.9%
flush), 10 mL, IV Push, BID

sodium chloride(sodium chloride 0.9%
flush), 10 mL, IV Push, As Directed,
PRN

temazepam, 15 mg= 1 cap, Oral, Daily at
bedtime, PRN

Home

acetaminophen(Tylenol 8 HR Arthritis Pain
650 mg oral tablet, extended release),
1300 mg= 2 tab, Oral, every 8 hr INT,
PRN

amLODIPine, 10 mg, Oral, Daily
aspirin(Aspirin Low Dose), 81 mg, Oral,
Daily

guaifenesin-pseudoephedrine(Mucinex D
Max Strength 1200 mg-120 mg oral tablet,
extended release), 1 tab, Oral, every 12
hr INT

losartan-hydrochlorothiazide

(losartan-hydrochlorothiazide 100 mg-25
mg oral tablet), 1 tab, Oral, Daily

magnesium glycinate, 500 mg, Oral, Daily
metoprolol(Metoprolol Tartrate), 25 mg,
Oral, BID

minoxidil(minoxidil 10 mg oral tablet), 5
mg= 0.5 tab, Oral, Daily

omeprazole, 40 mg, Oral, BID

terazosin(terazosin 10 mg oral capsule), 10
mg= 1 cap, Oral, Daily

Allergies

Codeine Sulfate (Unknown)

penicillin (Unknown)

Social History

Electronic Cigarette/Vaping

E-Cigarette Use:Never

Tobacco

Smoking tobacco use:Never (less than 100
in lifetime)

Total pack years:0

Lab Results

WBC: 5.54 cummk (05/31/2026 21:24)

RBC: 2.43 cummc Low (05/31/2026 21:24)

Hgb: 6.9 g/dL Critical (05/31/2026 21:24)

Hct: 21.6 % Critical (05/31/2026 21:24)

MCV: 88.9 fL (05/31/2026 21:24)

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MCH: 28.4 pg (05/31/2026 21:24)
MCHC: **31.9 g/dL** Low (05/31/2026 21:24)
RDW: **17.5 %** High (05/31/2026 21:24)
Platelets: **106 cummk** Low (05/31/2026 21:24)
MPV: NA (05/31/2026 21:24)
Neutro Auto: 69.7 % (05/31/2026 21:24)
Lymph Auto: **18.4 %** Low (05/31/2026 21:24)
Mono Auto: 9 % (05/31/2026 21:24)
Eos, Auto: 1.6 % (05/31/2026 21:24)
Basophil Auto: 0.4 % (05/31/2026 21:24)
Neutro Absolute: 3.86 cummk (05/31/2026 21:24)
Lymph Absolute: **1.02 cummk** Low (05/31/2026 21:24)
Mono Absolute: 0.5 cummk (05/31/2026 21:24)
Eos Absolute: 0.09 cummk (05/31/2026 21:24)
Baso Absolute: 0.02 cummk (05/31/2026 21:24)
NRBC Absolute: 0 cummk (05/31/2026 21:24)
Imm Gran Auto: 0.9 % (05/31/2026 21:24)
Prothrombin Time: **15.3 seconds** High (05/31/2026 21:24)
INR: 1.2 (05/31/2026 21:24)
Partial Thromboplastin Time: **41.1 seconds** High (05/31/2026 21:24)
Sodium Level: **133 mmol/L** Low (05/31/2026 21:24)
Potassium Level: 3.7 mmol/L (05/31/2026 21:24)
Chloride Level: 100 mmol/L (05/31/2026 21:24)
CO2: 24 mmol/L (05/31/2026 21:24)
Anion Gap: 9 mmol/L (05/31/2026 21:24)
BUN: **43 mg/dL** High (05/31/2026 21:24)
Creatinine Level: **2.06 mg/dL** High (05/31/2026 21:24)
Estimated Creatinine Clearance: 18.15 mL/min (05/31/2026 22:04)
Alk Phos: 88 U/L (05/31/2026 21:24)
ALT: **13 U/L** Low (05/31/2026 21:24)
AST: **14 U/L** Low (05/31/2026 21:24)
Glucose Level: **133 mg/dL** High (05/31/2026 21:24)
eGFR CKD-EPI: 23 (05/31/2026 21:24)
Calcium Level: **8 mg/dL** Low (05/31/2026 21:24)
Protein Total: 6.4 g/dL (05/31/2026 21:24)
Albumin Level: **2.6 g/dL** Low (05/31/2026 21:24)
Bilirubin Total: 0.4 mg/dL (05/31/2026 21:24)
Bilirubin Direct: 0.12 mg/dL (05/31/2026 21:24)

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21:24)
Magnesium Level: 2.2 mg/dL (05/31/2026 21:24)
21:24)
Troponin High Sensitivity: 149 ng/L Critical (05/31/2026 23:46)
Troponin High Sensitivity: 134 ng/L Critical (05/31/2026 21:24)
ProBNP: 4421 pg/mL High (05/31/2026 21:24)
T4 Free: 1 ng/mL (05/31/2026 21:24)
TSH: 1.894 uIU/mL (05/31/2026 21:24)
Procalcitonin: 0.16 ng/mL (05/31/2026 21:24)
UA Color: YELLOW (05/31/2026 21:41)
UA Appear: CLEAR (05/31/2026 21:41)
UA pH: 6.0 (05/31/2026 21:41)
UA Spec Grav: 1.010 (05/31/2026 21:41)
UA Glucose: NEGATIVE (05/31/2026 21:41)
UA Bili: NEGATIVE (05/31/2026 21:41)
UA Ketones: NEGATIVE (05/31/2026 21:41)
UA Blood: NEGATIVE (05/31/2026 21:41)
UA Protein: NEGATIVE (05/31/2026 21:41)
UA Urobilinogen: 0.2 (05/31/2026 21:41)
UA Nitrite: NEGATIVE (05/31/2026 21:41)
UA Leuk Est: 1+ Abnormal (05/31/2026 21:41)
UA WBC: 0-2 (05/31/2026 21:41)
UA RBC: None Seen (05/31/2026 21:41)
UA Bacteria: Trace Abnormal (05/31/2026 21:41)
UA Squam Epithelial: 0-5 (05/31/2026 21:41)
Influenza A PCR: NEGATIVE (05/31/2026 21:24)
Influenza B PCR: NEGATIVE (05/31/2026 21:24)
RSV PCR: NEGATIVE (05/31/2026 21:24)
Coronavirus SARS-CoV-2 (COVID-19): NEGATIVE (05/31/2026 21:24)
Transfusion Unit Number: END
W091026245515 (06/01/2026 11:12)
Transfusion Unit Number: START
W091026245515 (06/01/2026 10:17)
Transfusion Unit Number: END
W091026218585 (06/01/2026 09:06)
Transfusion Unit Number: START
W091026218585 (06/01/2026 05:15)
Transfusion Started: 06/01/2026 10:17 (06/01/2026 10:17)
Transfusion Started: 06/01/2026 05:15 (06/01/2026 05:15)
Transfusion Ended: 06/01/2026 11:12 (06/01/2026 11:12)
Transfusion Ended: 06/01/2026 09:06 (06/01/2026 09:06)
Blood Product: E0336 RBC CPD AS1 500 LR

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(06/01/2026 11:12)
Blood Product: E0336 RBC CPD AS1 500 LR
(06/01/2026 10:17)
Blood Product: E0336 RBC CPD AS1 500 LR
(06/01/2026 09:06)
Blood Product: E0336 RBC CPD AS1 500 LR
(06/01/2026 05:15)
Division: 00 (06/01/2026 11:12)
Division: 00 (06/01/2026 10:17)
Division: 00 (06/01/2026 09:06)
Division: 00 (06/01/2026 05:15)
Blood Product Expiration: 06/24/2026 23:59
(06/01/2026 10:17)
Blood Product Expiration: 06/24/2026 23:59
(06/01/2026 05:15)
Donor Blood Type: O POS (06/01/2026 10:17)
Donor Blood Type: O POS (06/01/2026 05:15)
Donation Type: Volunteer homologous
(allogeneic) donor (06/01/2026 11:12)
Donation Type: Volunteer homologous
(allogeneic) donor (06/01/2026 10:17)
Donation Type: Volunteer homologous
(allogeneic) donor (06/01/2026 09:06)
Donation Type: Volunteer homologous
(allogeneic) donor (06/01/2026 05:15)
Pre-Transfusion Checks: Transfusion order
reviewed, Informed consent verified,
Pre-transfusion medications given: None
ordered, IV Site:: Central Vein, Transfusion
education provided, Filter tubing used
(06/01/2026 10:17)
Pre-Transfusion Checks: Transfusion order
reviewed, Informed consent verified,
Pre-transfusion medications given: None
ordered, IV Site:: Central Vein, Transfusion
education provided, Filter tubing used
(06/01/2026 05:15)
User ID: Rogers Goodwin, Andrea
(06/01/2026 11:12)
User ID: Rogers Goodwin, Andrea
(06/01/2026 10:17)
User ID: Rogers Goodwin, Andrea
(06/01/2026 09:06)
User ID: Joyce, Molly (06/01/2026 05:15)
Donor Center Code: W0910 (06/01/2026
10:17)
Donor Center Code: W0910 (06/01/2026
05:15)
Transfusion Reaction Occurred: No
(06/01/2026 11:12)
Transfusion Reaction Occurred: No
(06/01/2026 09:06)

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Packed Red Blood Cells Volume: 326 mL
(06/01/2026 11:12)
Packed Red Blood Cells Volume: 500 mL
(06/01/2026 09:06)
ABO/Rh Type: O Pos (06/01/2026 02:16)
Antibody Screen Gel: Negative ABSC
(06/01/2026 02:16)
RBC Product Ready: Done (06/01/2026 02:16)
Crossmatch - Computer: Computer XM OK
(06/01/2026 02:16)
Crossmatch - Computer: Computer XM OK
(06/01/2026 02:16)
TRANSFUSED: TRANSFUSED (06/01/2026
11:12)
TRANSFUSED: TRANSFUSED (06/01/2026
09:06)
Lactic Acid Art POCT: 0.9 mmol/L (05/31/2026
21:35)
Collected?: Yes (05/31/2026 21:24)
Collected?: Yes (05/31/2026 21:24)
Collected?: Yes (05/31/2026 21:24)
Collected?: Yes (05/31/2026 21:24)
Positive Count: A Abnormal (05/31/2026
21:24)

Diagnostic Results

XR Chest 1 View Frontal

06/01/26 07:14:28

IMPRESSION:

1. Right internal jugular Infuse-a-Port is in good position with tip at the atrial caval junction.
2. Stable cardiomegaly.
3. Mild pulmonary vascular congestion.

Signed By: Barboza, MD, Jodi

CT Abdomen and Pelvis w/o Contrast

06/01/26 01:03:19

IMPRESSION:

1. No definite acute intra-abdominal or pelvic abnormality.
2. Colonic diverticulosis. There are multiple diverticula from the transverse colon to the sigmoid colon.
3. Hysterectomy.

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4. No obvious metastasis within the limits of a noncontrast exam. No enlarged or necrotic lymph nodes in the abdomen or pelvis.

5. Third spacing due to fluid overload. This includes small bilateral pleural effusions, mild ascites, and subcutaneous anasarca.

6. Cardiomegaly.

All CT scans are performed using dose optimization techniques as appropriate to a performed exam including automated exposure control and/or standardized protocols for targeted exams where dose is matched to indication/reason for exam/patient size.

Signed By: Samarawickrama, MD, Dilan

Electronically Signed on 06/01/26 02:10 PM

Hinson MD, Andrew

Electronically Signed on 06/01/26 11:33 AM

Meziere, APRN, Crystal